

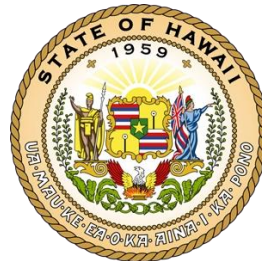
Hawai'i AHEAD Steering Committee

Tuesday, April 7, 2026

Jack Lewin, MD - Administrator, State Health Planning and Development Agency

Senior Advisor to Governor Josh Green on Healthcare Innovation, Hawai'i Department of Health

Joy Soares, MPA - AHEAD Project Director, Med-QUEST Division, Hawai'i Department of Health



Disclaimer

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Agenda

Hospital Global Budget Methodology

Presenter: Jon Fujii, Med-QUEST Division

Rural Health Transformation Program

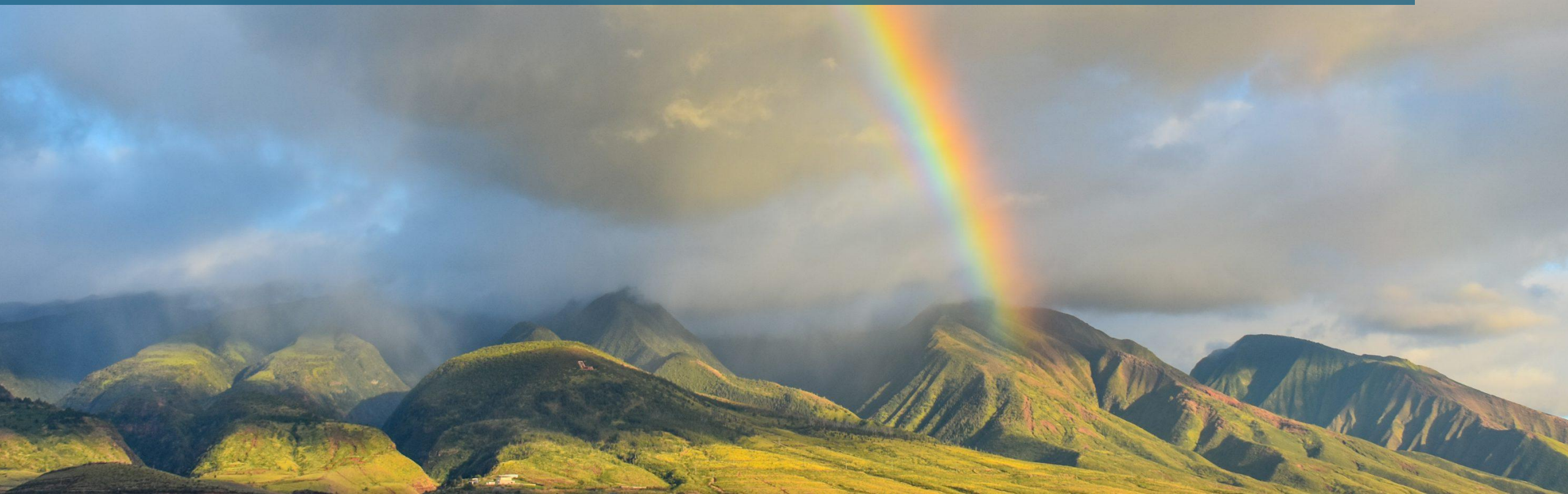
Presenter: Jack Lewin, MD SHPDA

AHEAD General updates, Milestones, and Timeline

Presenter: Jonas Yee SHPDA

Presentation:

Hospital Global Budget Methodology Updates



Changing How We Pay for Care

Jon Fujii

*Healthcare Services Branch Administrator, Med-QUEST
Division*



Med-QUEST Division, DHS

$$\text{VALUE} = \frac{\text{QUALITY} \quad \checkmark}{\text{COST} \quad \$} = \frac{\text{OUTCOMES PATIENT + EXPERIENCE}}{\text{DIRECT COSTS + INDIRECT COSTS}}$$

The New World

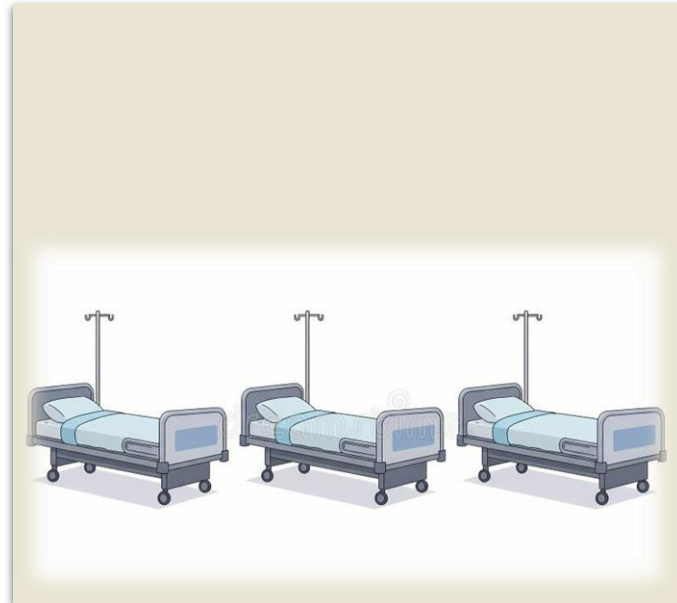
	Volume-Based	Value-Based
Payment	Fee-for-Service	Outcome Based
Incentives	Pass-A-Tube-Get-A-Payment	Keep-Em-Healthy-And-Make-A-Living
Focus	Episodes	Populations
Role of the	Interaction on	Team-Based Case

Value-Based Care

Current Model → Future Model



The current model continues to reward volume.



Phase I: Quality as a major revenue stream



Phase II: Hospital Global Budgets and aligned incentives



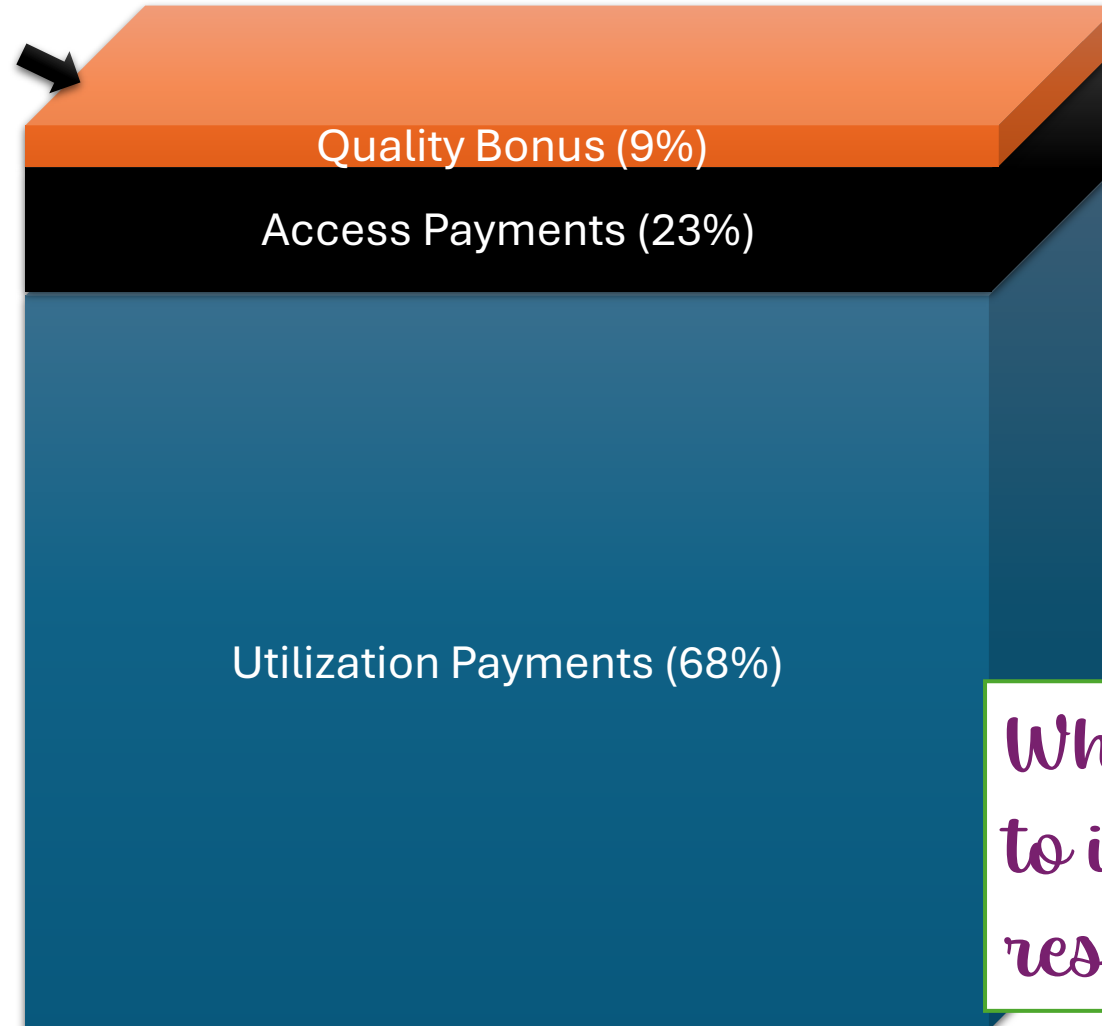
Medicaid Hospital Global Budgets

Structure and considerations for lookback analysis

Current Model (2025 Approximates)



Does increasing payout here...



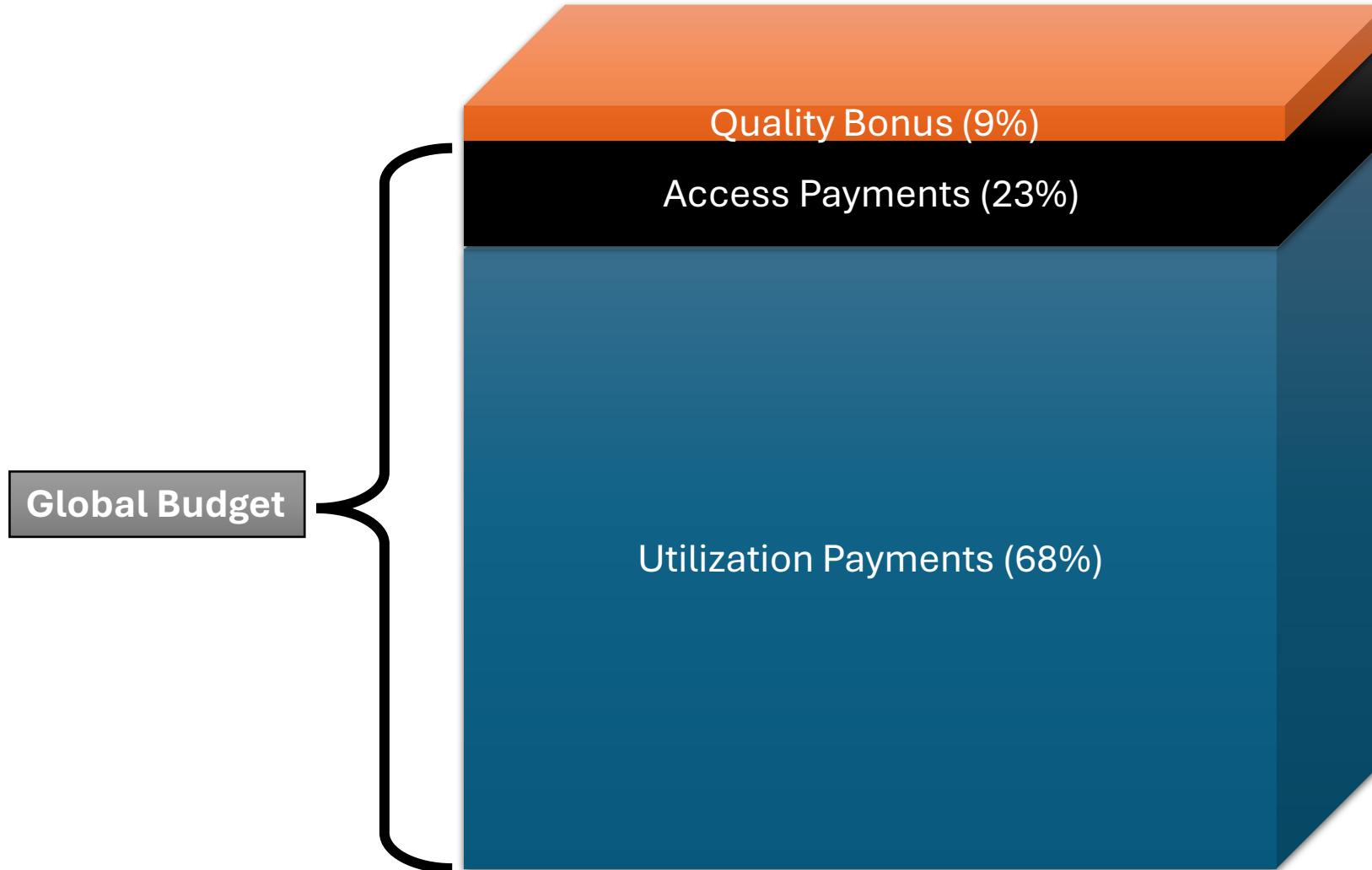
Result in a decrease in payout here?

Where does it “pay off” to invest my time and resources?

Hospital Global Budget Aligned Initiatives



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Hospital Global Budget Aligned Initiatives



Does increasing payout here...



Does not decrease payouts here

+ decreases costs/expenses that were previously cutting into profit margin

It can “pay off” to invest in the P4P program!



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Medicaid Global Budget Scenarios & AHEAD Implications

MQD & Milliman

Initial lookback analysis

Lookback Analysis



+
Does increasing payout here...
+
+

+
Does not decrease payouts
here
+
+ decreases costs/expenses
that were previously cutting
into profit margin +

Global Budget

Utilization Payments

Quality Bonus
Access Payments

This Analysis



Initial Lookback Simulation

Historic data from 2017 used as the baseline year.

Used to mock a HGB model from 2019-2024. Comparison to actual revenue.

Separate Medicaid, Medicare, and combined.



Model Adjustments

Applied

- Annual Payment Adjustment (APA)
- Demographic Adjustment (DA)
- Market Shift Adjustment (MSA)
- Medicare-only FFS AHEAD Adjustments (social risk adjustment SRA), effectiveness adjustment (EA), community improvement bonus (CIB), TCOC performance adjustment, Transformation Incentive Adjustment (TIA), CAH quality adjustment)
- Medicare-only Sequestration

Not Applied

- Outlier Adjustment
- Service Line Adjustment (SLA)

Additional adjustments (like HRSN, race-ethnicity, ESL, etc.) are possible as data becomes available.



Model Features

“Market shift floors” to support smaller hospitals on Oahu and all Neighbor Island hospitals.

- Does not penalize smaller hospitals that lose patient volumes due to market shift.
- These protective floors increased the HGB by \$57M.

Model Trend (expected year-over-year % increase in rates)

- Currently, we backed into the modeled 3.6% trend.
- The requested trend from CMS will be higher.

“Business as usual!” Without any change in care delivery.

- Improving quality will reduce hospital costs.
- Reductions in avoidable utilization will improve net revenue.

No Phase In Period (Part HGB/Part FFS)

- MQD has a proposal to stabilize hospital revenue under AHEAD despite HR1. Full HGB provides the “greatest up-front revenue.”
- Simpler administration for everyone.



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3.6% Trend



Statewide, revenue improved by 4.6%. 6 of 13 neighbor island hospitals, and 11 out of 20 hospitals (statewide) achieved net increases.



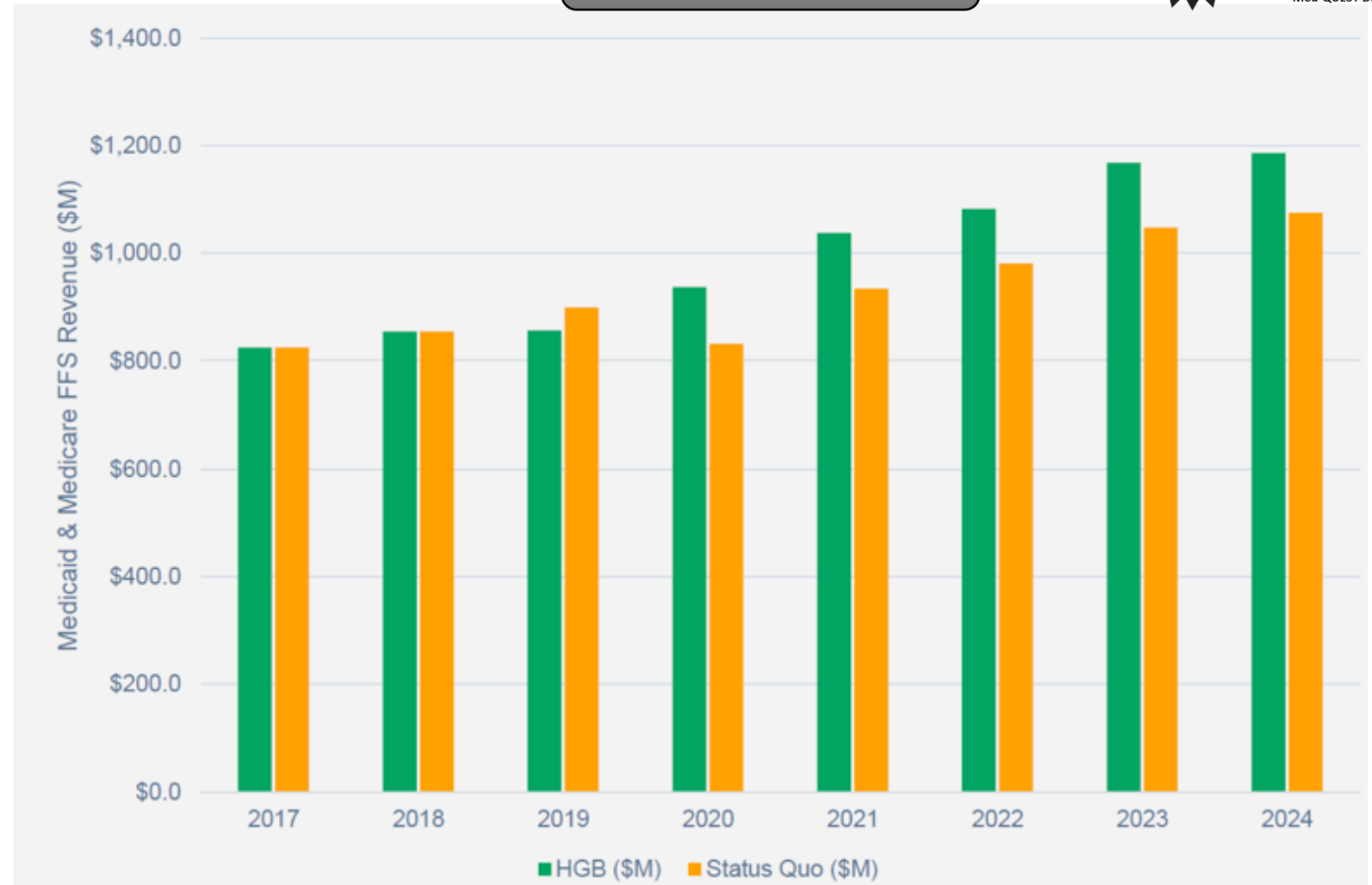
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2.5-3.0% Trend



Statewide, revenue improved by 13.0%. All neighbor island hospitals, and 18 out of 19 hospitals (in total) achieved net increases.

Blended Trend



Statewide, revenue improved by 8.5%. 15 out of 20 hospitals achieved net increases. Operating margin increased by 1.9% (2019-2024) [2.2% between 2023-2024].



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Expected Change	FFS Model	HGB Model
Increase		
Decrease		
Not Sure (?)/No Impact		



Most hospitals fared better! 15 out of 20 hospitals had a ++ outcome in aggregate.



Net operating margin improved by ~2% (55% improvement!)



The market shift floors added \$57M of revenue into the HGB.



HGBs are good to start during a “high trend” period. We are in a high trend period now! 😊



Quality improvements can improve all these outcomes significantly.



MQD has a “revenue stabilizing” proposal to CMS to mitigate impacts of HR1 for AHEAD hospitals

- Hospital-Specific Lookback Analyses (2017-2024) – distributed by the end of April 2026
 - Two Hospital CFO learning sessions to walk through individual spreadsheets, identify cost saving opportunities within AHEAD
 - Additional modeling or forecasting needs/feedback
- Meeting with CMS – CMCS & CMMI – on hospital revenue stabilization proposal is scheduled in May 2026
- Separate but related – Hospital utilization payments are impacted by APR-DRG. APR-DRG re-basing modeling is completed. Discussions with HAH and hospitals are coming up.
 - APR-DRG re-basing will also impact HGB base model



Rural Health Transformation Program

What is RHTP?

- A CMS program designed to support rural communities transform the healthcare delivery ecosystem.
- \$189 Million Dollars (per year) awarded to the State of Hawai'i
- Divided into 6 initiatives (**RHIN**, RICA, Pili Ola, Home Run, Rural Respite, **RVBI**)

Why it Matters for AHEAD

- Rural providers often lack the scale, infrastructure, and financial flexibility needed to participate successfully in value-based payment arrangements such as AHEAD without additional support.

What Hawai'i Requested

- **Initiative 6, RVBI** – This competitive fund will enable rural providers to adopt care models and succeed under the AHEAD model by financing local value-based innovations. This may include support for quality reporting, advanced analytics, practice transformation coaching, hiring staff.

Status

- SHPDA was awarded RHTP funding. Collaborating with Med-QUEST and DOH on planning and procurements.
- Working closely with the Governors office to coordinate with other initiatives

Initiative 1: **Rural Health Information Network**

- Electronic Health Records Upgrades
- Last Mile Network Infrastructure Solutions
- Care Quality Information Exchange
- Statewide Analytics & Technical Assistance Hub
- Closed Loop Referral System and Care Coordination Hub
- Dual Eligible Data Dashboard Hub

Initiative 6: Rural Value Based Initiative

The AHEAD Readiness Fund

Rural Hospitals

**Community
Health Centers**

**Independent
Providers**

What is a CIN?



State RHTP Application Available Online:

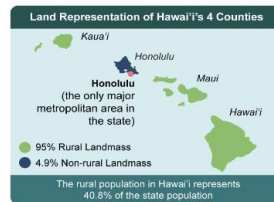
engage.hawaii.gov/rhtp/

State of Hawai'i CMS Rural Health Transformation Plan Project Narrative

1. Rural Health Needs and Target Population

A. Rural Demographics

Hawai'i, an isolated archipelago of Pacific Islands, is the most remote populated landmass on earth. Seven of the Hawaiian island are inhabited with a total population of 1.45 million. Hawai'i is a five to six-hour flight from California and a five to six-hour time zone difference from the nation's capital.¹ The only major metropolitan area in the state (Honolulu) is located on the island of O'ahu. All other areas of O'ahu and all other islands (known as the "neighbor islands") are rural. The state's rural areas are defined in Hawai'i Revised Statutes §1B-1².



Hawai'i is divided into four counties, including Honolulu, Maui, Kaua'i, and Hawai'i (for this application, Kalawao County—a county of less than 90 people and administered by the Hawai'i Department of Health—is considered part of Maui County). The majority (95%) of Hawai'i's land area is considered rural, representing 5,999 square miles. The rurality of Hawai'i

b. Subrecipient Awardees (Initiatives) Initiative 1: Rural Health Information Network (RHIN)

Name of Subrecipient: State Health Planning and Development Agency (SHPDA)

Period of Performance: FY26 – FY31

Scope of Work: SHPDA will lead the establishment and statewide rollout of the Rural Health Information Network (RHIN) initiative, which modernizes Hawai'i's rural health data infrastructure, in coordination with the State's Medicaid office (Med-QUEST Division). SHPDA will procure and manage contracts for electronic health record (EHR) onboarding, in-facility network installations, and interoperability integration for statewide health information exchange. Deliverables include implementation of four RHIN sub-hubs: the Care Quality Information Exchange (CQIE), Community Care Coordination Hub (C-Hub), Analytics & Technical Assistance Hub, and Duals Data Dashboard & Support Hub (DDDASH). Outcomes will include increased data interoperability, expanded closed-loop referrals, and measurable improvements in dual eligible integration, telehealth usage among providers, and value-based readiness.

Method of Oversight: The RHTP Project Director and Oversight Team will oversee SHPDA's implementation of the initiative. RHIN initiative leaders will meet with the RHTP Project Director and relevant members of the Oversight Team monthly. The lead agency (SHPDA) will provide quarterly progress reports and expenditure documentation. RHIN initiative leaders (or their designated representative) will participate in monthly RHTP Stakeholder Advisory Committee meetings, ensuring coordination among initiatives and community leaders, transparency in conduct, and real-time stakeholder feedback on implementation.

Itemized Budget and Justification:

Initiative	Rural Health Information Network (RHIN)				
	BP 1	BP 2	BP 3	BP 4	BP 5
A. Personnel	\$249,696	\$249,696	\$249,696	\$249,696	\$249,696
B. Fringe	\$159,954	\$159,954	\$159,954	\$159,954	\$159,954
C. Travel	\$3,250	\$3,250	\$3,250	\$3,250	\$3,250

b. Subrecipient Awardees (Initiatives) Initiative 1: Rural Health Information Network (RHIN)

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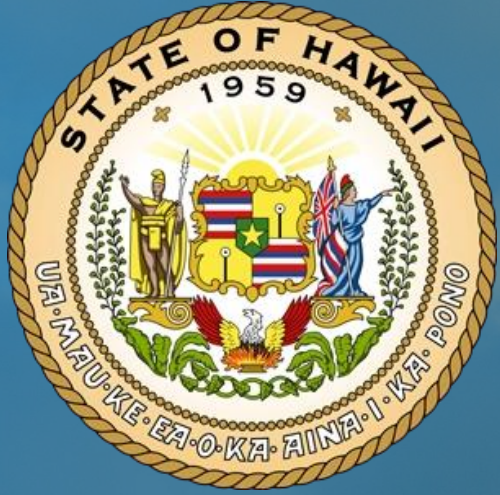
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AHEAD Timeline

Activity	2024 - 2026		2027	2028	2029	2030	2031-2035	
AHEAD Model Period	Pre-Implementation/Planning Period			Performance Period				
Hospital Global Budgets				Medicare FFS HGBs Starts 1/1/28				
				Medicaid HGB Non-Risk Year Starts	Medicaid HGB Starts 1/1/229			
					Commercial HGB (at least one payer) Starts 1/1/29			
Primary Care AHEAD				Medicaid Advance Primary Care Program Starts 1/1/28				
				Medicare and Commercial Primary Care AHEAD Starts 1/1/28				
RHTP			2026-2031 (Year 1: 1/1/26 – 09/30/26)					

We are here



QUESTIONS? MAHALO

